



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ليلى حافظ سيد احمد
المحافظة : القاهرة
الحى / المدينة : عين شمس
رقم البطاقة : 26305072101042 رقم التليفون : 01155419105
العنوان :
العمر (تاريخ الميلاد) : 07-05-1963 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 24-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ليلي حافظ سيد احمد National ID: 26305072101042	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

????, ??? Patient ID: 10110224013350 DOB: 07-May-1963 Date: 16-Nov-2024

Patient Lastname: حافظ

Patient Firstname: ليلى

HIS Patient ID: 10110224013350

RIS Patient ID: 362014

Date of Birth: 07/05/1963

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (16/11/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" Heterogeneous tracer uptake is noted over the lumbar vertebrae, more evident at LV5 and LV5?? Marked
degenerative changes?? Otherwise for further CT correlation.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion: -

" Non-uniform tracer uptake is noted over the lumbar vertebrae for further CT correlation.

" Otherwise, no definite evidence of active metastatic osseous deposits could be seen all over the rest of the
surveyed skeleton.

Much Obligated

Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====

جمهورية مصر العربية
بطاقة تحقيق الشخصية

اليلي
حافظ سيد احمد
٢٨ اخرش الحاج رفاعى الامام الغزالى
امبابه - الجيزه

٢٠٢٢/٠٦/٠٧ ٢٦٣.٥.٧٢١.١.٤٢

JJ8386287

٢٠٢٢/٠٦ ٢٦٣.٥.٧٢١.١.٤٢

ربة منزل

انثى مسلمة متزوجة
مصطفى محمد توفيق اسماعيل

البطاقة سارية حتى ٢٠٢٩/٠٦/٠٧

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Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : ليلى حافظ سيد احمد

Sex:

3- Medical number:
Female

83214199

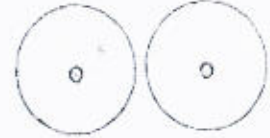
Old Med :

D4 Med :

1464-2024

2-ID number:

Patient Age : 61 Y



Family history	Family history +ve	Phone	Phone 01155419105	BMI	BMI	REASON OF MDT	REASON OF MDT POST OPERATIVE
marital status	Marital Status MARRIED	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT 16/09/2024
Co morbidity	Co morbidity htn	Menopausal status	Menopausal Status POSTMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

It breast multiple malignant masses , SUSPICIOUS
MICROCALCIFICATIONS EXTENDING INTO 4 QUADRANTS
AREA 12CM level i indeterminate axillary LN

Mets Workup :

ct cap: rt upper lobe tiny pulmonary nodules likelu benign
D6 vertebral body ill defined small lytic lesion

BX IDC	ER 8/8	PR 8/8	Her 2 Neu 1	SISH	Ki67 10
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T 1m	N INDER TMINANT	M
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Sonomammography

PATIENT HISTORY

It mastetcomy+SLNB:IDC GII
free nipple ,areolar, deep margin
PT3 (8CM) P N3 LN 10/11
FOR BONE SCAN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

- 1- ADJUVANT ACX4 TAXOLX12
- 2- PORT
- 3- ADJUVANT AI

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY [Signature]	RADIOLOGY	RADIOLOGY [Signature]
SURGERY	SURGERY [Signature]	PATHOLOGY	PATHOLOGY [Signature]
Psychiatric	Psychiatric [Signature]	Nutrition	Nutrition [Signature]



Patient Name	: الأستاذة / ليلي حافظ سيد احمد	Request No.	: 13007246
Patient No.	: 26305072101042	Request Date	: 30/07/2024
Path No.	: 2638/24	Verification Date	: 19/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 61 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:

Left breast mass.

Nature of the specimen:

Left mastectomy, sentenil lymph node biopsy and axillary clearence specimens:collected at 30/7/2024.

Gross Examination:

Three specimens were received :

-Sentenil lymph node biopsy specimen :Multiple fibrofatty tissue peices collectively measured 3x3x3 cm enclosing 4 nodules up to 1.3x0.8 cm.

-Axillary clearence specimen:

Multiple fibrofatty tissue peices collectively measured 6x6x6 cm enclosing 7 nodules up to 2x2 cm.

-Mastectomy specimen measured 27x24x5 cm with covering skin elipse measured 19x18 cm

• Tumor extent

o Number: An ill defined greyish white rubbery to firm beaded mass.

o Size:8x7x5 cm. ^{T3}

• Distance from margins:1 cm from deep margin and 0.2 cm from skin.

• Frozen section:

-Positive SLNs (4/4).

Microscopic examination:

• Histological type:Invasive duct carcinoma with marked desmoplasia and necrosis.

• Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+2).

• DCIS:Focal.

• Angioinvasion :Positive.

Verified by :



Patient Name : الأستاذ / أيلى حافظ سيد احمد	Request No. : 13004227
Patient No. : 26305072101042	Request Date : 06/06/2024
Path No. : 1947/24	Verification Date : 12/06/2024
Sex : Female	Financial Category : برنامج شارب - مياثره
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youssef

Pathology Report

Clinical Data: Left breast mass at 2 o'clock (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected at 06/06/2024.

Gross Examination:

- Multiple grayish white tissue cores up to 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (2+2+2) Grade II.
- * Ductal carcinoma in situ: Not present.
- * Perineural invasion: Not present.
- * Angioinvasion: Not present.
- * Microcalcifications: Not present.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

For IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Amal Ahmed Hareedy,

Dr. Rufaida Abdullah.

Verified by :

Printing Date : 24/06/2024 09:17:12

Page 1 of 1

Printing By : Mrs. Inas Hareedy

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم 987 - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Laila Hafez Sayed Ahmed	Date : 31/07/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 62 Year(s)
Medical ID: 1964/2024	Pat. ID: 54541

Bilateral high resolution sonography revealed:

Clinical summary: Post left MRM & ALAND, No available radiological data.

Findings:

Right breast.

- Preserved fibroglandular densities.
- 12 o'clock middle third, circumscribed oval parallel homogenous benign looking lesion measuring 15 x 10 mm with absent internal vascularity on color Doppler study
- No suspicious solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- A large seroma measuring 15 x 1.5 cm with septations and thick wall, surrounding inflammatory changes are noted.
- No suspicious masses
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 3): 12 o'clock probably benign solid mass lesion mostly fibroadenoma.

Left mastectomy bed large complicated seroma with surrounding inflammatory changes , yet no suspicious masses or pathological lymph nodes, for US follow up after adequate treatment

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
PM 5:56 2025/07/31

Patient Name: Laila Hafez Sayed Ahmed	Date : 04/06/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 61 Year(s)
Medical ID: 26305072101042	Pat. ID: 54541

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Left breast palpable mass

Report:

- ACR b: scattered fibroglandular parenchyma
- **Left Breast:**
 - Asymmetrical global density associated with ill-defined interconnected spiculated dense lesions with overlying grouped of suspicious microcalcification are seen at retro-areola region extending into the four breast quadrants. The area including the asymmetry, masses and calcifications is extending over an area measuring 90x75x120mm
 - Ultrasound revealed multiple ill-defined irregular heterogeneous hypoechoic and isoechoic interconnected masses reaching the nipple with subsequent retracted nipple.
 - One indeterminate likely infiltrated level 1 axillary lymph node measuring 8mm in TS shows focal cortical thickness measures 3mm.
 - No supra or infraclavicular lymph node.
- **Right Breast:**
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Left breast multiple malignant masses forming an infiltrative process and level I indeterminate axillary lymphadenopathy for histopathological assessment (BIRADS 5)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,

Dr. Eman Abdel Razik, MSc, FEBR

Dr. Amira H Radwan MD,

Breast Imaging consultant,

NCI, Cairo University

04/06/2024 12:42 PM

Patient Name: Laila Hafez Sayed Ahmed	Date : 07/04/2025
Exam : C.T CHEST	Age : 62 Year(s)
Medical ID: 1964/2024	Pat. ID: 54541

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Compared to last CT dated 16-12-2024

Findings:

- Evidence of left mastectomy with stationary course of the operative bed oblong shaped seroma.
- Clear both lung fields apart from few scattered bilateral thin atelectatic bands.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Small hiatus hernia still noted.
- Normal appearance of the chest wall.
- Upper abdominal cuts are unremarkable.

OPINION:

- Evidence of left mastectomy with stationary course of the operative bed oblong shaped seroma.
- Clear both lung fields apart from few scattered bilateral thin atelectatic bands.

Dr. Ibrahim Elsedawy